

PA Status Relay

Product Requirements Document: Net New Build

Build name: Practice-Side PA Status Relay

Owner: Jillian Krebsbach, Natalie Walker, Chris Wonziack, Lee McDonald

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1. PROBLEM

Oncology PA coordinators managing prior authorizations for infused, buy-and-bill drugs have no real-time visibility into case status — leaving coordinators checking payer portals manually, patients calling repeatedly for updates, and practices carrying financial risk on drug inventory they've already purchased but cannot yet administer.

Supporting Context

- AMA 2023: Physicians submit an average of 43 PA requests/week; 95% report care delays from PA (prior authorization) requirements.
- CoverMyMeds data: PA status notifications cut time to prescription pickup by 2.2 days, proving downstream impact.
- Practices assume cash-flow risk on buy-and-bill drug inventory (e.g., \$180K/year IV oncology drugs) pending PA approval; delays and denials translate directly to financial harm.

1a. Opportunity

Oncology practices need a purpose-built tool for the medical-benefit PA pathway — a gap no major competitor currently fills. Solving this creates a scalable beachhead in a market of ~2,600 US oncology practices with projected >50% demand growth through 2040.

Market Opportunity

- ~2,600 addressable US oncology practices; ~16,700 physicians ordering PA-requiring drugs; ~1M patients on systemic therapy annually.
 - Competitive gap: leading PA status tools serve pharmacy benefit only — no major player addresses medical/infusion benefit authorization.
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1b. Users & Needs

Primary users: PA/Infusion Coordinators — managing high-volume PA caseloads with fragmented tools, manual portal checks, and no single system of record.

Secondary users: Internal reviewers (product, engineering, design) evaluating the demo for feasibility and investment readiness. Tertiary: oncology patients receiving infused systemic therapy (simulated in MVP).

Key User Needs

- As a coordinator, I need to log and update PA statuses with required metadata enforced so I can reduce manual effort and avoid downstream rework.
- As a coordinator, I need to preview patient-facing messages before simulated delivery so I can confirm accuracy and consent compliance.
- As a patient, I need plain-language updates on my authorization status so I can make treatment and life decisions without calling the office.

2. PROPOSED SOLUTION

PA Status Relay is a web-based workflow tool that gives oncology coordinators a single dashboard to log, track, and relay prior authorization status updates for infused, buy-and-bill drugs. Coordinators create mock cases, walk through each status transition with required metadata enforced, preview patient-facing messages, and generate an immutable audit trail. The MVP is a 5-day, Supabase-backed demo with mock data — no live EHR or payer integrations — designed to validate the core workflow and generate internal buy-in for a production build.

2a. Value Proposition

PA coordinators who struggle with fragmented, portal-dependent authorization tracking use PA Status Relay, a workflow dashboard, to log status updates, preview patient communications, and maintain a clean audit trail in one place. Unlike manual portal checks and disconnected notes, it enforces required metadata at every transition and delivers plain-language patient updates — reducing per-case handling time and eliminating the guesswork that delays treatment starts.

2b. Top 3 MVP Value Props

- The Vitamin: A single, structured case list with status tracking and audit trail — the table-stakes foundation coordinators currently manage across portals and sticky notes.
- The Painkiller: Required-metadata enforcement and an immutable audit log that stops duplicate work, prevents incomplete submissions, and removes debate during stakeholder reviews.
- The Steroid: Plain-language patient message previews with consent gating — coordinators see exactly what patients receive before it goes out, eliminating inbound "where's my authorization" calls.

2c. Goals & Non-Goals

Goals

- Validate the buy-and-bill PA relay workflow in a single 5-day sprint with 5 scripted demo scenarios.
- Demonstrate measurable admin time savings: median coordinator status update ≤ 90 seconds vs. a ~5–10 minute manual baseline.
- Generate internal stakeholder buy-in: $\geq 80\%$ of reviewers rate the demo "useful/feasible."
- Confirm that patient-facing message previews are jargon-free and consent-gated.

Non-Goals

- No live EHR or payer integrations in MVP — integration work begins in v2.
- No production SMS or message delivery — patient messages are UI previews only.
- No PHI handling or compliance certification — all demo data is mock and non-identifiable.
- No appeal/Denied → Submitted loop — deferred to v2 with payer-specific logic.

2d. Success Metrics

Goal	Signal	Metric	Target
End-to-end flow completes	All 5 scenarios run error-free	% scenarios completed per session	100%
Coordinator time savings	Faster than manual baseline	Median time: open case → audit entry	≤90s
Patient message clarity	No jargon or denial rationale	% messages passing plain-language review	≥95%
Stakeholder buy-in	Positive internal feedback	% reviewers rating "useful/feasible"	≥80%

3. REQUIREMENTS

User Journey 1: Coordinator — Case Triage & Status Logging

Context: The primary demo flow. Coordinators need fast, discoverable actions to log status changes with required metadata enforced and see results immediately in the audit trail.

Sub-journey: Case management

[P0] User can view a list of mock PA cases with key metadata (patient name, status, timestamp, consent flag).

[P0] User can search, sort, and filter cases by status, patient name, or date.

[P0] User can create a new case with required fields (patient name, consent flag) and optional metadata.

[P1] User can edit an existing case's metadata.

Sub-journey: Status logging

[P0] User can select only valid status transitions from the current state (invalid transitions are disabled).

[P0] User is blocked from completing a transition if required metadata is missing (e.g., doc link for Submitted), with a specific inline error.

[P0] User sees an immutable audit entry within 500ms of a successful status change, including: new status, previous status, actor, timestamp, and reason code.

[P1] User can provide an optional reason code on any transition; reason code is required for

Denied and Info Request transitions.

Sub-journey: Patient message preview

[P0] User sees a plain-language message preview modal on every status change when consent flag is TRUE.

[P0] User sees a disabled send state with a consent CTA when consent flag is FALSE; a "message suppressed — no consent" event is logged to the audit trail.

[P1] User can edit the templated message text in the preview modal; edited messages are flagged as "custom message" in the audit trail.

User Journey 2: Coordinator / Reviewer — Audit Trail Review

Context: Reviewers and coordinators need to verify demo fidelity and QA completeness. The audit trail is the primary evidence of workflow integrity.

Sub-journey: Audit inspection

[P0] User can view a chronological, immutable audit trail per case with: timestamp, actor, action, previous status, new status, reason code, and message sent (Y/N).

[P0] User cannot edit or delete audit trail entries — no edit or delete controls are rendered.

[P0] User can filter the audit trail by action type, actor, or date range without a page reload.

[P1] User can export the audit trail as a CSV with case ID and export date in the filename.

Sub-journey: Demo controls

[P0] User can Reset a case to its seeded baseline state (with confirmation); reset event is logged as a demo_event, not a status transition.

[P0] User can Clone a case, creating an independent copy with an empty audit trail and status: New Order.

[P1] User can replay a scenario from Clone without affecting the original case's audit trail.

4. APPENDIX

State Machine (9 statuses, MVP scope)

Statuses: new_order · needs_documentation · submitted · pending_review · info_request · peer_to_peer · approved · denied · closed

Valid transitions:

New Order → Needs Documentation, Submitted
 Needs Documentation → Submitted
 Submitted → Pending Review, Needs Documentation (return, amber path)
 Pending Review → Approved, Denied, Info Request, Peer-to-Peer
 Info Request → Pending Review, Submitted (re-submit)
 Peer-to-Peer → Pending Review only (MVP constraint)
 Approved → Closed (requires appointment link)
 Denied → Closed (requires next-step note; no appeal in MVP)
 Closed → Terminal state

MVP constraints:

- No Denied → Submitted (appeal path deferred to v2)
- Peer-to-Peer → Pending Review only (direct P2P resolution deferred to v2)
- Demo Re-open writes a demo_event row only — not a status transition

Patient-Friendly Status Mapping (Appendix C)

Internal Status	Patient-Facing Message
New Order	We've received your treatment order. We'll update you as we make progress.
Needs Documentation	Your care team is preparing everything needed for insurance review.
Submitted	Your PA request has been submitted and is under insurance review.
Pending Review	Your insurance is reviewing your request. We'll contact you when there's a decision.
Info Request	We're sending additional information to your insurer.
Peer-to-Peer	A clinical discussion has been requested by your insurance provider.
Approved	Your treatment is approved. Scheduling will contact you next.
Denied	Your insurance did not approve your request. Your care team will discuss next steps.

Internal Status	Patient-Facing Message
Closed	Your authorization case is complete. For questions, contact [office #].

Competitive Gap Summary

Competitor	Coverage	Gap vs. This Product
CoverMyMeds (McKesson)	Pharmacy-benefit PA status texts	No medical-benefit coverage; no infused/buy-and-bill flow
D2 Solutions	Real-time PA texts via pharma hub services	Hub-services model, not practice-facing; no medical-benefit
SmithRx / Prescriptive	PBM member portals with live PA tracker	Pharmacy-benefit only; consumer-facing
Infinx Patient Access	PA alerts pushed to EMR and patient portal	Not patient-facing status relay for infused drugs specifically
Clearwave / Curogram	Two-way texting and reminders; 98% SMS open	No PA-specific functionality

Tech Stack & Architecture

- Backend/DB: Supabase (auth + Postgres + RLS). Free tier supports unlimited team members — one owner creates the project and invites collaborators.
- Frontend: Figma design system → Claude Code component generation → Codex for state machine and logic-heavy pieces.
- Architecture: Single-page app. No external integrations. Mock data only. Sized for internal demo use (~2–4 concurrent users).
- No PHI. No production SMS. No EHR or payer integration in MVP.

Audit Schema (written on every status transition):

```
{ case_id, from_status, to_status, user_id, timestamp, reason_code, doc_link, message_sent }
```

Demo controls (Reset, Clone, Re-open) write to a separate demo_event table — not the audit trail — to keep QA export clean.

5-Day Sprint Sequence

Day 1: Supabase setup + schema + seed data + auth flow + Case List UI

Day 2: Case create/edit + status logging + transition validation + state machine UI

Day 3: Audit trail + message preview modal + consent flag logic + demo controls

Day 4: Polish + error states + accessibility + end-to-end QA + hosting confirmed

Day 5: Internal run-through + bug fixes + reviewer access shared

Key dependencies before Day 1:

- Figma tokens + Case List / Case Details frames locked (Day 0)
- Supabase project created, team invites sent and confirmed
- Status enum and message copy (Appendix C) locked — schema changes after Day 1 are expensive

Research Sources

- AMA 2023 Prior Authorization Physician Survey
- CoverMyMeds PA Status Notification Impact Data
- Oncology Clinical Trials Product Evaluation Framework (July 2026, project folder)
- Market sizing: ~2,600 US oncology practices; ~16,700 hematology/medical oncologists; ~1M Americans on systemic therapy annually; >50% projected demand increase by 2040

End of Document · v0.4 Condensed · July 2026